

Expert Witness Letter Templates

Rambabu Vadlamudi — EB-1A Petition

These templates are for expert witness letters supporting Ram's EB-1A petition.

Customize each letter for the specific expert's credentials and relationship to Ram's work.

Template 1 — Laboratory Director (for use after HealthTrackRx pilot launches)

Best for: Lab directors at pilot sites (HealthTrackRx, Principle Health Systems)

Strongest EB-1A criteria served: C5 (original contributions), C8 (leading role)

[Date]

U.S. Citizenship and Immigration Services
[Service Center Address]

RE: Expert Letter in Support of EB-1A Extraordinary Ability Petition for Rambabu Vadlamudi

Dear Officer:

I am writing in strong support of the EB-1A Extraordinary Ability petition for Rambabu Vadlamudi. I am [Name], [Title] of [Laboratory Name], a CLIA-certified independent clinical laboratory located in [City, State]. My laboratory processes approximately [X] molecular diagnostic and pharmacogenomics claims monthly. I have [X] years of experience in laboratory operations and clinical billing, and I am recognized in my field through [credentials/affiliations].

The Problem Mr. Vadlamudi's Work Addresses

Independent clinical laboratories in the United States face a systemic billing crisis that threatens the viability of our industry. As documented by JAMA Network Open (Georgetown University, 2025, n=29,919), independent laboratories face 2.76 times higher odds of claim denial compared to hospital-based laboratories — a disparity that reflects structural regulatory complexity, not differences in clinical quality. The molecular diagnostics denial rate has been documented at 35.3% across 20 million claims (XiFin

2024 Payor Denial Impact Report), the highest of any healthcare specialty. My laboratory [specific quantified impact statement — e.g., “has experienced X denials per month in our molecular diagnostic line, representing \$X in monthly preventable revenue loss”].

Why Mr. Vadlamudi’s DGP Architecture Is an Original and Major Contribution

Mr. Vadlamudi invented the DGP (Deterministic-Generative-Predictive) Clinical Revenue Architecture — a novel three-layer AI system that represents, to my knowledge, the first published architecture purpose-built for independent laboratory molecular diagnostic claim recovery. The architecture combines:

- A 847-rule deterministic policy engine covering all LCD, NCD, and MolDX compliance requirements — the regulatory framework that governs claims at laboratories like mine
- An LLM-based clinical reasoning layer that generates compliant appeal briefs in under 90 seconds
- A machine learning layer that predicts and prevents denials before claim submission

In my [X] years in laboratory operations, I have not encountered any existing technology platform that addresses the MolDX program requirements, DEX Z-code compliance, and CPT 81400-81479 genomic sequencing documentation requirements that govern our billing. Mr. Vadlamudi’s architecture fills a gap that has cost the independent laboratory industry an estimated \$10-12 billion annually in preventable revenue losses.

[PILOT RESULTS PARAGRAPH — Insert after pilot launch]: Following the deployment of Mr. Vadlamudi’s platform at our laboratory in [Month Year], we observed [specific outcomes: X% reduction in denial rate, \$X in recovered revenue, X% reduction in appeal preparation time]. These results demonstrate the practical impact of Mr. Vadlamudi’s original contribution on laboratory operations.

National Importance

Independent laboratories serve populations that hospital-based labs often do not reach — rural communities, addiction medicine clinics, specialty oncology practices. The PAMA 2027 reimbursement schedule, which projects cumulative Medicare cuts of up to 45% by 2029, poses an existential threat to our industry. More than 50% of independent laboratory directors reported considering consolidation or sale if PAMA cuts take effect (ACLA 2024). AI-driven claim recovery that reduces the 35.3% denial rate represents a direct economic lifeline for laboratories that serve these communities.

Conclusion

In my professional opinion, Rambabu Vadlamudi's DGP architecture represents an extraordinary and original contribution to healthcare technology. His work addresses a quantified, nationally significant problem that no existing platform solves. The United States benefits greatly from his continued presence and ongoing development of this technology.

I strongly recommend approval of his EB-1A petition.

Sincerely,

[Name] [Title] [Laboratory Name] [CLIA Number] [Contact Information]

Template 2 — Healthcare AI Expert (Academic or Industry)

Best for: Faculty at UT Southwestern, Baylor Scott & White, Georgetown (JAMA 2025 study PI); Google Cloud healthcare team; JAMA/XiFin researchers

Strongest EB-1A criteria served: C5 (original contributions), national importance argument

[Date]

U.S. Citizenship and Immigration Services
[Service Center Address]

RE: Expert Letter in Support of EB-1A Extraordinary Ability Petition for Rambabu Vadlamudi

Dear Officer:

I am writing in strong support of the EB-1A Extraordinary Ability petition for Rambabu Vadlamudi. I am [Name], [Title/Position] at [Institution/Company]. My expertise is in [healthcare AI / clinical informatics / molecular diagnostics / healthcare revenue cycle management], and I have [X] years of experience in this field. [Brief credential statement: publications, positions, recognition.]

I am independent of Mr. Vadlamudi and have no personal or financial relationship with him or his company. I offer this assessment based on my expertise and knowledge of the current state of the field.

National Context: The Independent Laboratory Billing Problem

The United States independent clinical laboratory sector faces a systemic reimbursement crisis. [Reference the expert's own published work if

applicable — e.g., “As documented in our 2025 JAMA Network Open analysis of 29,919 NGS claims,”] molecular diagnostic claim denial rates reach 35.3% — the highest across all healthcare specialties (XiFin 2024, 20M+ claims). Independent laboratories face 2.76× higher denial odds than hospital-based laboratories. Despite 50-80.7% appeal win rates, 65% of denied claims are never appealed due to the complexity of clinical documentation requirements and the resource constraints of independent laboratory billing departments.

This represents an estimated \$10-12 billion in annual preventable revenue loss and threatens the viability of 7,000+ CLIA-certified independent laboratories — facilities that disproportionately serve rural communities, specialty practices, and underserved populations.

The Originality of the DGP Architecture

Mr. Vadlamudi invented the DGP (Deterministic-Generative-Predictive) Clinical Revenue Architecture. In my assessment as an expert in [healthcare AI / clinical informatics / this domain], this represents an original and significant contribution to the field for the following reasons:

First, the architecture’s domain of application — healthcare administrative automation for laboratory claim recovery — is distinct from all prior published work on hybrid AI in healthcare. Existing published hybrid AI healthcare architectures address clinical diagnosis and treatment recommendation. Mr. Vadlamudi’s architecture is the first to apply this three-layer design to the administrative domain of laboratory billing regulatory compliance.

Second, the deterministic-first design principle — encoding 847 compliance rules from LCD/NCD/MolDX/DEX Z-code policies in a rule engine that operates upstream of any generative AI component — addresses a genuine technical problem with LLM-only approaches. Generative models hallucinate on specific coverage policy determinations. Mr. Vadlamudi’s architectural design eliminates this failure mode by ensuring the LLM generates clinical narrative grounded in pre-resolved, deterministic compliance determinations.

Third, the architecture achieves native compliance with Texas SB 1188 and TRAIGA (HB 149), demonstrating technical leadership in responsible AI governance in the healthcare revenue domain. To my knowledge, it is the first clinical revenue AI system designed for compliance with these frameworks from inception.

Why the United States Benefits

No existing commercially available or published AI system addresses the specific regulatory complexity of independent laboratory molecular diagnostic billing at the scale and domain specificity that Mr. Vadlamudi’s

architecture provides. [If applicable: “Our research documented the scale of this problem; Mr. Vadlamudi’s architecture represents the technological solution.”]

The \$10–12 billion annual revenue loss from preventable claim denials represents a direct cost to the US healthcare infrastructure. AI-driven recovery of even a fraction of this loss directly supports the financial viability of independent laboratories that are essential to rural and specialty healthcare access.

Conclusion

Based on my expertise in [field], I assess Rambabu Vadlamudi’s DGP architecture as an extraordinary original contribution of major significance to healthcare technology. His work is nationally important, technically distinguished, and without peer in its application domain. I strongly recommend approval of his EB-1A petition.

Sincerely,

[Name] [Title] [Institution] [Contact Information]

Expert Witness Recruitment Strategy

Highest Priority Targets

1. **Georgetown University PI** (JAMA Network Open 2025 study on NGS denial rates) — Ram’s work directly addresses the problem their paper documented; natural partnership. Contact via Georgetown Medical Center.
2. **XiFin Principal Analyst** (2024 Payor Denial Impact Report) — their data underpins Ram’s national importance argument. Contact via XiFin’s research team.
3. **HealthTrackRx Lab Director** — after pilot launch (Q2 2026), outcomes data makes this the most powerful letter.
4. **Google Cloud Healthcare Team** — Ardia uses GCP with HIPAA BAA; Google Cloud can attest to architectural significance of the GCP implementation.
5. **UT Southwestern Medical Center** (healthcare informatics faculty) — DFW-based, credible academic institution.

Approach Template (Email to Expert)

Subject: Expert Letter Request — EB-1A Petition — Healthcare AI for Independent Laboratory Billing

Dear [Name],

I am Rambabu Vadlamudi, Founder of Ardia Health Labs in Argyle, TX. I am the inventor of the DGP (Deterministic-Generative-Predictive) Clinical Revenue Architecture — a hybrid AI system for independent laboratory claim denial recovery.

I have read [their paper / am familiar with your work] and believe your expertise in [field] makes you uniquely qualified to assess the significance of my contribution. I am preparing an EB-1A Extraordinary Ability petition and am seeking an expert witness letter.

[Your paper / Your company's research] documented [specific statistic from their work that Ram's architecture addresses]. My architecture directly addresses this problem. I believe a letter from you — assessing whether my architectural contribution is original and nationally significant — would be highly valuable.

I would be happy to provide you with a full technical description of the architecture, the white paper, and a draft letter template for your review and editing. This would require approximately 1-2 hours of your time.

Would you be willing to speak with me briefly about this?

Best regards, Rambabu Vadlamudi
ram.vadlamudi@ardiahealthlabs.com (469) 679-3334